CHILDREN'S MEMORIAL HOSPITAL — EMERGENCY DEPARTMENT

88 Pediatric Plaza, Seattle, WA 98101 | ED Direct: (206) 555-0900

EMERGENCY DEPARTMENT NOTE

Patient:	Aiden James Park	**MRN:**	CMH-ED-56231
DOB:	04/12/2015 (Age 8)	**SSN:**	701-88-4420
Date/Time:	12/01/2023 | 02:14 AM	**Triage Level:**	2 (Emergent)
Address:	742 Evergreen Terrace, Apt 2, Seattle, WA 98103	**Phone:**	(206) 555-7721
Parent/Guardian:	Soo-Yeon Park (mother)	**Parent DOB:**	06/30/1982
Emergency Contact:	Jin Park (father) — (206) 555-4490	**Insurance:**	Premera Blue Cross #PBC-339921
ED Physician:	Dr. Carlos Reyes, MD	**Nurse:**	Amanda Fletcher, RN

CHIEF COMPLAINT

Acute onset wheezing and shortness of breath, worsening for 4 hours.

HISTORY OF PRESENT ILLNESS

Aiden is an 8-year-old male with a known history of moderate persistent asthma who presents with his mother at 2:14 AM with a 4-hour history of worsening respiratory distress. Mother reports he was in his usual state of health until approximately 10 PM when he began coughing. He had exposure to a cat at a friend's sleepover this evening (known allergen). He used his albuterol rescue inhaler (2 puffs) at 10:30 PM and again at 12:30 AM with minimal relief. No fever. No recent URI. No vomiting. Last ate at 8 PM (dinner). Current PEFR on arrival: 48% predicted.

PAST MEDICAL HISTORY

Moderate persistent asthma since age 4. Two prior ED visits (2021, 2022) — both treated with nebulizers and discharged. No prior intubations. No ICU admissions. Eczema (mild). Seasonal allergic rhinitis.

Allergies: Cat dander (bronchospasm); Amoxicillin (rash)

Immunizations: Up to date per school records. Flu vaccine received 10/2023.

Current Medications: Fluticasone 44 mcg inhaled BID (controller); Albuterol PRN rescue; Cetirizine 5 mg daily

PHYSICAL EXAMINATION

Vitals: T 37.1°C | HR 122 | RR 32 | SpO2 91% on RA | Wt 28 kg

General: Anxious, sitting upright, visible use of accessory muscles.

Respiratory: Moderate intercostal and subcostal retractions. Diffuse expiratory wheezing bilaterally. Decreased air entry at bases. No stridor.

Cardiovascular: Tachycardic. Regular rhythm. No murmur.

TREATMENT IN ED

02:20 AM — Supplemental O2 via nasal cannula at 2 L/min, SpO2 improved to 95%.

02:25 AM — Albuterol 2.5 mg + Ipratropium 0.5 mg nebulized (first dose).

02:45 AM — Albuterol 2.5 mg nebulized (second dose). SpO2 97%, RR 24, wheezing improved.

03:00 AM — Prednisolone 1 mg/kg PO (28 mg given).

03:30 AM — Albuterol 2.5 mg nebulized (third dose). Significant improvement. PEFR 74% predicted.

ASSESSMENT & DISPOSITION

Moderate acute asthma exacerbation, triggered by cat allergen exposure. Responded well to bronchodilator therapy and steroids. Discharged home at 04:15 AM in stable condition with SpO2 98% on room air.

DISCHARGE INSTRUCTIONS

1. Albuterol MDI 2–4 puffs q4h PRN x 5 days.
2. Prednisolone 1 mg/kg (28 mg) daily x 5 days (prescription given).
3. Continue fluticasone as scheduled.
4. Strict avoidance of cat exposure.
5. Follow up with pediatrician Dr. Hannah Nguyen, MD within 48 hours — (206) 555-4400.
6. Return to ED immediately if: SpO2 <92%, severe retractions, inability to speak in sentences, or no relief with rescue inhaler.

ED Physician: Dr. Carlos Reyes, MD | NPI: 7788990011 | Signed: 12/01/2023 04:18 AM